

Prof. Dr. Qamruzzaman Chowdhury

MBBS, DMRT, FCPS

Senior Consultant, Oncology

Bangladesh Specialized Hospital.

BMDC Reg: A14404



HN ID : H12104156856

Name : NAZRIN NAHER

Age : 55Y 5M 9D

Gender : Female



Visit ID : C12606956708

Visit Date : 10 Jun 2026

Visit Time : 10:28 AM

Diagnosis

- Carcinoma of left lung. Post Ablation. Post CT> Tab Osimertinib (since Sept/21 to Nov/25) > Suspected Liver & Vertebral (L2) metastases- Static. On Follow up

Clinical History

Ref: Mr Shahriar, Beacon/ Dr. Abu Zakaria, Germany

Histopathology: On 21.03.21, Modern, Left lung lesion, CT guided FNAC, adenocarcinoma

On 24.03.21, Anowara, Lung lesion left, FNA, Non small cell carcinoma

On 01.12.22, FNAC- Atypia with undetermined significance

On 07.12.22, Anowara, Thyroid nodule- Follicular lesion of undetermined significance

Immunohistochemistry: On 12.04.21, Anowara, TTF1-Positive; P63- Negative; Primary lung adenocarcinoma

Molecular/ Gene Study: On 20.05.21, Praava, **Positive for-** EGFR (Exon 21 L858R)

Negative for- ROS1, EGFR T790M, EML4-ALK, MET, BRAF V600

Surgery: X

Radiotherapy: Microablation on 22/03/21

Chemotherapy: H/O taking Osimertinib for 1.5 months > Pemetrexed- Carboplatin, **Cycle 6 completed on 30.08.21**

History

Marker Study

CEA : On 17.07.2021 = <1.73, On 06.12.2021 = <1.73, On 22.2.22=0.9> On 04.02.2023= <1.73, On 09.08.2023= 0.76>On 18.12.23=0.95> On 06.02.2025= 0.72> On 02.11.2025= <1.73

Investigation Notes:

On 17.03.21, CT chest: Small mass lesion in left upper lobe, bronchial neoplasm (about 3 cm)

On 24.03.21, Colour doppler study of hepatic arterial and venous system: Marked (Grade 3) diffuse fatty infiltration of liver with two small benign intrahepatic cysts (17 x 16 mm, 19 x 18 mm), Normal study of IVC, Portal and hepatic venous system. On 08.05.21, PET CT- Bilateral centrimetric cervical level IA (right SUVMax-3.54 & left SUVmax-4.61), bilateral cervical II (right SUVmax-5.15 & Left SUVMax-5.49) and bilateral multiple subcentrimetric sized cervical level V (right SUVmax-3.07 and left SUVmax-3.53) suggest malignancy (mets), post ablated centrally necrosed lung lesion about 34.2x29.3 (Suvmax-2.84) in antero-apical segment of left upper lobe abutting left chest wall by fibrosis band likely to be post ablated effect, bilateral centrimetric sized axillary LN (SUVmax-6.19 right and left SUVmax-6.09) are noted suggest malignancy (mets), bilateral iliac group of LN (right SUVmax-4.93 and left SUVmax- 5.24) suggest mets, bilateral iguinal LN (right SUVmax-2.88 and left SUVmax-2.56) suggest malignancy

On 17.07.21, CT Chest- Centered in left upper lobe. Measurement-3.4 x 2.8 x 3.6 cm. The mass is invading the adjacent costal pleura with associated minimal pleural reaction as pleural thickening. The intercostal

Electronic Signature

Prof. Dr. Qamruzzaman Chowdhury

Chamber: 11:00AM - 6:00PM (Daily except Friday and Public Holiday), Report Viewing: After 1st visit within 7 days single visit

Free , Every report showing and follow up visit needs appointment. Please contact 10633

Printing Date & Time : 15-08- 2026 05:03 PM

Software By :: Mysoft Limited

Prof. Dr. Qamruzzaman Chowdhury

MBBS, DMRT, FCPS

Senior Consultant, Oncology

Bangladesh Specialized Hospital.

BMDC Reg: A14404



HN ID : H12104156856

Name : NAZRIN NAHER

Age : 55Y 5M 9D

Gender : Female



Visit ID : C12606956708

Visit Date : 10 Jun 2026

Visit Time : 10:28 AM

muscles minimally appear to be invaded at the inner margin. However no definite gross invasion seen. Central necrotic change is evident. Peripheral fibrotic bands are noticed as a post treatment response. Few prominent axillary lymphnodes are seen in both sides. Single node in both sides are mildly enhancing, may suggest infiltration. Maximum diameter 1.0 cm.

CT Neck- Multiple prominent lymphnodes are seen in both cervical chains. Involvement seen in almost all the levels. The nodes are predominantly discrete in nature. The most marked prominent nodes are identified in level II with maximum diameter 1.3 cm. Few of the nodes showing moderate enhancement.

CT WA- Multiple small lesions seen in both lobes of liver. Intense contrast enhancement (at least 4 lesions are identified) seen with predominant peripheral nodular pattern seen in both lobes of liver. The largest lesion measured 1.8 x 1.9 cm. Fatty change seen in liver. Liver is mildly enlarged in size (19 cm). Small non-enhancing cystic lesion seen in left lobe of liver measuring about 1.7 x 1.7 cm. Centimetric enhancing nodes in both iliac and inguinal stations --- Metastatic. Few reactive nodes in paraaortic station

On 04.10.21, PET CT- Previously seen soft tissue mass at anterior segment of upper lobe of lung shows interval changes with metabolic and morphologic regression (SUVmax-1.3 vs prior 2.8, measures about 31x18mm vs prior 34x29mm) with fibrotic band suggests therapy response, previously seen multiple FDG avid bilateral cervical, axillary, inguinal and iliac LN(SUVmax-1.2 vs 5.2 on left side) show interval change with regression - suggest therapy response, hepatomegaly with a small cyst at segment II with mild uptake (SUVmax-2)

On 06.12.21, USG WA: Grade-I fatty change in liver, small cyst (20.5x17.9mm) at segment II of liver, angiomyolipoma (9.8x9.2mm) at lower pole of left kidney

CXR- Nodular opacity in left upper zone

On 15.2.22, USG of WA-Mild generalized fatty infiltration of liver with a tiny hepatic cyst about 20.1x14.3mm.; Features suggestive of an angiomyolipoma of left kidney(about 9.3x7.6 mm)

CXR- A rounded dense opacity is noted in left upper zone.

On 06.07.22, PET CT: Relatively stable spiculated soft tissue density mass 3.1x1.7 cm is noted in left upper lobe, SUV max 2.5, radiating spicules and fibrotic scarring with traction bronchiectasis is noted in adjacent upper lobe. Few 4-5 in number ill defined well enhancing nodular lesion appearing isodense to liver parenchyma.

On 29.11.22, PET CT- Heterogeneously hypoenhancing spiculated soft tissue lesion with low grade metabolic activity in the apicoposterior segment of left lung upper lobe. No mediastinal and hilar LNpathy. An FDG avid subcentrimetric hypodense nodule with speck of calcification in right lobe of thyroid.

On 02.12.22, USG KUB- A well defined hyper echoic foci with appreciable internal vascularity in the lower pole of the left kidney.

On 04.02.23, USG W/A: Cystic lesion (25.2x20.0mm) in left lobe of liver. Angiomyolipoma (12.3x11.8mm) in left kidney- size increased.CXR: NAD

On 12.08.23, CT scan of chest- Residual lesion in left lung with plate atelectic change. Enhancing lesion in liver suggests mets (21x19) mm- suggest mets USG WA- Simple hepatic cyst in left lobe

Electronic Signature

Prof. Dr. Qamruzzaman Chowdhury

Chamber: 11:00AM - 6:00PM (Daily except Friday and Public Holiday), Report Viewing: After 1st visit within 7 days single visit Free , Every report showing and follow up visit needs appointment.Please contact 10633

Printing Date & Time : 15-08- 2026 05:03 PM

Software By :: Mysoft Limited

Prof. Dr. Qamruzzaman Chowdhury

MBBS, DMRT, FCPS

Senior Consultant, Oncology

Bangladesh Specialized Hospital.

BMDC Reg: A14404



HN ID : H12104156856

Name : NAZRIN NAHER

Age : 55Y 5M 9D

Gender : Female



Visit ID : C12606956708

Visit Date : 10 Jun 2026

Visit Time : 10:28 AM

On 17/08/23, MRI of Liver: Multiple small areas of altered signal intensity in both the lobes of liver- suspicious for metastasis. A simple hepatic cyst in left lobe of liver. Few subcentimeter lymphnodes in paraaortic region- suspicious nodes. Small enhancing lesion in body of L2 vertebra on left side- suspicious for metastasis. Bilateral few small renal cystic lesions. Comment: No change in size and number of the lesions of liver as compared with previous PET CT dated 06.07.22. In that scan no abnormal FDG uptake was seen. Those were commented as multiple hemangiomas. Small bony lesion in body of L2 vertebra this bony lesion was also present in previous PET CT dated 06.07.22.

On 19.12.23, PET CT- Marginal regression in size and relatively stable in metabolic activity of spiculated soft tissue density mass in left upper lobe. Relatively stable well enhancing nodular lesions appearing isodense to liver parenchyma in delayed phase in segment VIII of liver- likely hemangioma. Stable size of FDG non avid cortical cyst in lower pole of left kidney- Bosniak type IV.

On 18.04.24, USG WA- Fatty liver Gr-I with hepatic cyst, angio-myo-lipoma in left kidney CXR PA- Pulmonary inflammatory lesion right

On 10.09.24, CXR: Haziness is noted in both lower zone - ? pneumonitis. Unfolding of aorta and aortic knuckle calcification is noted. Hump is noted in right dome of diaphragm, bony thorax revealed degenerative changes in visible dorsal vertebrae. USG WA: Mild generalized fatty infiltration of liver with hepatic cyst. tiny angiomyolipoma and a cortical cyst in left kidney.

On 04.11.24, PET-CT: The left upper lobe density is partially obscured from adjacent scarring/volume loss of presumed posttreatment change. Mild low-grade uptake within the nodular component is present, stable/slightly lower than written report of prior external FDG PET study in 23 (images not available for review). Whilst the low-grade uptake may reflect post treatment inflammation, small volume residual disease is difficult to completely exclude and ongoing surveillance suggested. No hypermetabolic nodal or distant metastases. Mild focal uptake at right thyroid upper pole.

On 11.11.25, PET CT: No local recurrence. Focal hypermetabolic lesion (SUVmax 9.74) with perilesional edema in right parietal lobe of brain.

On 15.11.25, MRI Brain: No focal lesion within brain parenchyma. Focal cortical atrophy of right parietal lobe near vertex.

On 05.06.26, CBC & others: WNL, Creatinine: 59.17umol/L, SGPT: 75.91, SGOT: 66.92

On 06.06.26, CT chest with WA: Few fibrotic bands in left upper lobe - suggesting old healed lesions. Few small soft tissue nodules in both the lobes thyroid. Marked diffuse fatty changes in liver. Small thin walled cystic lesion in IV of left lobe of liver - suggesting simple hepatic cyst. No significant change observed except fatty changes in liver, as compared with last PET CT done about 7 months back - suggesting stable disease.

Finalization

Present Plan :

20/11/25: considering no radiological evidence of disease on PET scan, MRI of brain and H/O osimertinib for 04 years- decided to stop osimertinib for time being.

Electronic Signature

Prof. Dr. Qamruzzaman Chowdhury

Chamber: 11:00AM - 6:00PM (Daily except Friday and Public Holiday), Report Viewing: After 1st visit within 7 days single visit

Free , Every report showing and follow up visit needs appointment. Please contact 10633

Printing Date & Time : 15-08- 2026 05:03 PM

Software By :: Mysoft Limited

Prof. Dr. Qamruzzaman Chowdhury

MBBS, DMRT, FCPS

Senior Consultant, Oncology

Bangladesh Specialized Hospital.

BMDC Reg: A14404



HN ID : H12104156856

Name : NAZRIN NAHER

Age : 55Y 5M 9D

Gender : Female



Visit ID : C12606956708

Visit Date : 10 Jun 2026

Visit Time : 10:28 AM

Advice

- Follow up after 06 months with patient & reports. Please take appointment before visit.
- Consult with Prof Dr Masudur Rahman/ Dr. Baharul Alam Gastreenterologist.

Investigation Advised

- PET scan of whole body (Medinova, Dhanmodi / IBN sina, Dhanmondi/ Labaid Cancer Center, Greenroad/ Evercare hospital & Nuclear medicine dept of Dhaka MCH / BMU/ CMH/ Nuclear medicine institute, Savar/ INMAS, Mohakhali)
- Marker Study- CEA (ICDDR or BSH)
- CBC, S. Creatinine, Bilirubin, Alkaline Phosphatase

Electronic Signature

Prof. Dr. Qamruzzaman Chowdhury

Chamber: 11:00AM - 6:00PM (Daily except Friday and Public Holiday), Report Viewing: After 1st visit within 7 days single visit

Free , Every report showing and follow up visit needs appointment. Please contact 10633

Printing Date & Time : 15-08- 2026 05:03 PM

Software By :: Mysoft Limited